جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : فاتن حمامه محمد شحاته ممم
المحافظة : القاهره **الحي / المدينة :** الزيتون **العنوان :** مم
رقم البطاقة : 26004242300444 **رقم التليفون :** 01110471005 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 24-04-1960 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام تصوير مقطعي للصدر والبطن والحوض بالصبغة [11129] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنه و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: فاتن حمامه محمد شحاته ميم
National ID: 26004242300444

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Contrast-enhanced CT of chest, abdomen and pelvis.

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



DMS
تاكرب
زيلودا



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

07/03/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	MRN:	6246
Age:	65 Y	ID:	26004242300444
Menopaus; Status:	postm	Phone:	01110471005
NRS SCORE:	ps 3	Marital Status:	
BMI:		Family History:	
CO Morbidity:		Reason Of MDT:	mbc triple positive with recent DP

Clinical Data:

BX:

ER PR HER KI67

Sonomammography:

Mets Workup:

History:

postm ol lateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left er8/8 pr8/8 her2/3+ rec to rol herceptin infusions 10/2/2025 widespread oss lesions pt rec palliative rt to left hip j s j cis 5/5/2026 rd for new bone scan
pt rec 7 cycles herceptin 21 weeks taxol
bone scan on 18/6/2025 widespread osteoblastic and edenotic osseous lesions

Recommendations:

MDT Outcome & Plan

shift to Tykerb - Xeloda
Zometa monthly after dental evaluation

R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Name: فاتن حمامه محمد شحاته	
MR. : 6246	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 66-year-old individual with a history of bilateral breast cancer. The left breast tumor was estrogen-receptor positive (ER 8/8), progesterone-receptor positive (PR 8/8) and HER2-positive (3+). The right breast tumor was estrogen-receptor positive (ER 7/8), progesterone-receptor positive (PR 6/8) and HER2-negative. Initial treatment included taxane-based chemotherapy (taxol) and trastuzumab (Herceptin) with a regimen of 10 mg/m² every 2 weeks, followed by seven cycles of trastuzumab over 21 weeks and taxol. The patient subsequently received palliative radiotherapy to the right hip, but declined further radiotherapy due to prior treatment at the same site.

Pathology confirmed the hormone-receptor and HER2 status as recorded. Imaging with a bone scan on 18 June 2026 demonstrated widespread osteoblastic and sclerotic osseous lesions. Recent computed tomography (CT) scans show a stationary disease course, whereas the bone scan raises concern for progressive disease in the left mid-shaft femur, proximal tibia and fibula, with prior scans showing uptake in both femoral bones. The patient has not yet received the approved combination of capecitabine (Xeloda) and lapatinib (Tykerb) and has continued intravenous trastuzumab pending further MDT decision.

The MDT concluded that the disease is stationary for the new tumor marker CT. The plan is to continue intravenous trastuzumab and add femara, refer to urology for evaluation of hydronephrosis, administer zoledronic acid (Zometa) every three months, maintain the port-a-catheter, and perform an echocardiogram.

تحريرا في ,

التاريخ :	23-09-2026	**الوقت :**	

الطبيب المعالج :

يعتمد ,

Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m1183

اسم المريض	: فاتن حماسه محمد	النوع	: Female
السـن	: 65 Y	العنوان	: القاهرة
تاريخ اخذ العينة	:	تاريخ استلام العينة	: 30/07/2025 09:32
طالب التحليل	: مركز صحه المرأة	تاريخ التقرير	: 06/08/2025 10:25
عناية	: مركز صحه المرأة		

Primary Sample :

Paraffin Block

Test :

ER &PR & HER2 neu & Ki67 (ER&PR: Harvey scoring system -Her2neu : CAP scoring system)

Comment :

Technique :

sample recieved was treated by: - CONFIRM anti-estrogen (ER) (SP1) Rabbit monoclonal primary antibody.- CONFIRM anti-progesterone receptor(PR) (IE2)Rabbit monoclonal primary antibody.
Sample recieved was treated by : -VENTANA anti-HER2/neu (4B5) Rabbit monoclonal primary antibody using DAB Detection kit in ventana benchmark GX / ULTRA .
Sample recived was treated by :- CONFIRM anti Ki67 (30-9) Rabbeit monoclonal primary antibody.

Result :

RIGHT BREAST CORE :- *ER: strong nuclear staining of 60 % of tumour cells
PR: strong nuclear staining of 20 % of tumour cells
HER2/neu protein over expression : discontinuous membrane staining of tumor cells.
Ki67 :positive nuclear staining of 5 % of tumour cells
LEFT BREAST CORE :- *ER: strong nuclear staining of 80 % of tumour cells
PR: strong nuclear staining of 70 % of tumour cells
HER2/neu protein over expression : strong continuous membrane staining of > 10 % of tumor cells.
Ki67 :positive nuclear staining of about 15 % of tumour cells

Referal Lab :

Comment :

Conclusion :

RIGHT BREAST CORE :- *ER: POSITIVE (SCORE 7 / 8)
PR: POSITIVE (SCORE 6 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : NEGATIVE (SCORE 1+)
#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS 5 % .
LEFT BREAST CORE :- *ER: POSITIVE (SCORE 8 / 8)
PR: POSITIVE (SCORE 8 / 8)
HER2/NEU PROTEIN OVER EXPRESSION : POSITIVE (SCORE 3+)
#Ki67: THE PROLIFERATION INDEX OF THE TUMOUR IS ABOUT 15% .

Pathologist: د/ايناس رمزى

Lab Director: د/نريزا ماهر

وزارة الصحة
الإدارة المركزية للمعامل

19 El-sheikh Rehan Street, Abeideen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website http://www.healthlabs.net
email:pathologychl@yahoo.com
facebook: قسم الباثولوجى - الإدارة المركزية للمعامل

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Ministry of health
Central health laboratories
Pathology department

Histopathology report

Report Number: 214

سم المريض	:	فاتن حمامه محمد	النوع	:	Female
لســـن	:	65 Y	العنوان	:	القاهرة
تاريخ اخذ العينة	:		تاريخ استلام العينة	:	29/07/2025 10:17
طالب التحليل	:	مركز صحه المرأة	تاريخ التقرير	:	31/07/2025 11:45
عناية	:	مركز صحه المرأة			

Primary sample :

1- Left breast core biopsy
2- Right breast core biopsy

comment :

Gross examination :

1 - fragmented cores of greyish white tissue (T . I)
2 - fragmented cores of greyish white tissue (T . I)

Microscopic examination :

1 - Section examined revealed fibrocollagenous tissue infiltrated by malignant lobular cells arranged in nests & indian files pattern embeded in desmoplastic stroma .
2- Section examined revealed fibrocollagenous tissue infiltrated by strands of malignant lobular cells arranged in indian files pattern embeded in desmoplastic stroma.

Referal Lab :

Comment :

Diagnosis :

1- LEFT BREAST CORE BIOPSY :- THE PICTURE IS COMPATIBLE WITH INVASIVE LOBULAR CARCINOMA .
2- RIGHT BREAST CORE BIOPSY :- THE PICTURE IS COMPATIBLE WITH INVASIVE LOBULAR CARCINOMA .

Pathologist: DrJehan Aziz

Lab Director: دانريزا ماهر

19 El-sheikh Rehan Street, Abeideen, Cairo
Tel.(165)7948544 -7950096 Fax. (202)7948544
Mobile . 01144297292
website:http://www.healthlabs.net
email:pathologychl@yahoo.com
facebook: قسم الباثولوجى - الادارة المركزية للمعامل

يتم الاحتفاظ بأصل العينة لمدة 30 يوم من تاريخ استلامها

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Patient ID: 86435	Patient Name English: Faten Hamama Mohamed Shehata
Age: ٦٥ سنة, ٣ شهر	2025/07/28 11:48:03 AM :Report Date

Women's Health Outreach Program
Bilateral Full field Digital Mammography

Clinical summary: 65-year-old lady with negative family history complaining of right nipple retraction for two months.

Technique

Medio-lateral oblique and CC views High resolution ultrasound

Detailed Findings

Right breast

- Almost entirely fatty breast parenchyma (ACR density a).
- Upper central, middle third, irregular speculated high density mass lesion with surrounding architectural distortion and no suspicious clusters of micro calcifications.
- Thickened skin and coarsened trabeculae.
- **Ultrasound** demonstrated 12 o'clock, 5.6 mm deep to the skin, irregular antiparallel speculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo, posterior acoustic shadowing and moderate internal vascularity on color Doppler study measuring 40 x 22 mm. Such lesion is seen reaching the nipple through few dilated ducts reaching 7 mm in caliber that are seen with soft tissue material within as well as showing abnormal vascularity within.
- Thickened skin reaching 5.5 mm and coarsened trabeculae.
- Multiple (more than three) discreate pathological enlarged axillary; level 1; LNs are seen the largest measured 24 x 11 mm with 11 focal cortical thickness. Multiple seen at level 2, as well the largest measured 19 x 9 mm and few seen at level 3, largest measured 6 x 6 mm. No pathological supra-clavicular lymph nodes.

Left breast

- Almost entirely fatty breast parenchyma (ACR density a).
- UOQ, middle third, irregular high density mass lesion with micro-lobulated margins and surrounding architectural distortion yet with no suspicious clusters of micro calcifications.
- **Ultrasound** demonstrated 2 o'clock, 4 cm from nipple 1.5 mm deep to the skin, irregular antiparallel speculated heterogenous hypoechoic solid mass lesion with surrounding echogenic halo,

Mammogram
[[DOCTYPE:MAMMOGRAM]]

posterior acoustic shadowing and mild internal vascularity on color Doppler study measuring 15 x 6 mm.

- Normal skin/subcutaneous tissue and nipple/areolar complex.
- Two level 1 axillary indeterminate lymph nodes are noted, largest measured 9 x 6 mm with 3 mm cortical thickness. No level 2, 3 or supraclavicular pathological LNs.

Impression:

Right breast (BIRADS 5): Upper central, 12 o'clock locally advanced suspicious mass lesion with multiple ipsilateral enlarged pathological lymph nodes; levels 1,2 & 3. US-guided Tru-cut biopsy was conducted at same setting of the breast lesion.

Left breast (BIRADS 4c): UOQ, 2 o'clock, suspicious solid mass lesion with ipsilateral two indeterminate axillary (level 1) lymph nodes. US-guided Tru-cut biopsy was conducted at same setting of the breast lesion.

Recommendations:

Right breast: histopathological correlation.

Left breast: histopathological correlation.

Much obliged,

Dr. Amal Hegazi, M.Sc

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Patient's Name:	**Faten Hamama Mohammed Shehata**
Date:	**2026-05-05**
Referred by:	Prof.Dr. **CT CAP+C**
Hospital:	**Goustave Roussey International Egypt**

MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

As compared to the last CT examination dated 11/2/2026, the current examination revealed

I- CHEST:

- Stationary course regarding size of the previously noted right breast retro-areolar parenchymal distortion with two nodules largest is measuring 1.2cm and overlying skin thickness with stationary course regarding size and number of the ipsilateral enlarged axillary lymph nodes, the largest still measures about 13.5x9.5 mm as in previous study. **for further sono-mammography**
- Clear left breast and left axilla.
- No pulmonary nodules, masses or cavitary lesions.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Still noted enlarged cardiac size with atherosclerotic calcific foci of the aorta.
- Still noted bilateral elastofibroma dorsi.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- The gall bladder is well distended, average wall thickness with no stones.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or right back pressure changes.
    - Stationary moderate to marked left hydronephrosis and left hydroureter till vesico-ureteric junction with radiodense stone detected ?? attenuated distal end ?? reflux
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.

CT Scan
[[DOCTYPE:CT_SCAN]]

Patient's Name:	**Faten Hamama Mohammed Shehata**
Date:	**2026-05-05**
Referred by:	Prof.Dr. **CT CAP+C**
Hospital:	**Goustave Roussey International Egypt**

- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No abdominal or pelvic lymphadenopathy.
- Multiple pelvic calcific phleboliths are noted.
- No ascites.
- **The examined skeleton**
  - Almost stationary course as regards the multiple lytic and sclerotic bony metastasis along the axial and appendicular skeleton with stationary sclerotic lesions in lower end of sternum, D5 down to L2 , D8, D11 and L3 vertebral collapse and multiple expansile areas/lesions of ribs on both sided being evident on right 6th rib.
  - **For further bone scan.**

Opinion:

- **Stationary course since the last study with no denovo lesions.**

Dr. Mai Shaaban MD

Dr.Mai Shaaban

Dr.Mai Shaaban

CT Scan
[[DOCTYPE:CT_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة والتحاليل

٦٢٤٦

Name : Faten Hamama Mohammed Shehata	Date : 18-06-2026 09:00
Branch : Dokki	Medical No. : 396924
Ref.Physician Prof : Goustav Roussey Egypt	Ins.Company : تعاقد مستشفيات

Whole-Body Bone Scintigraphy Report

Indication: Skeletal staging for right breast carcinoma in a 66-year-old female patient; status post chemotherapy and radiotherapy; generalized bone pain.
Procedure: Anterior and posterior whole-body planar imaging after intravenous Tc-99m MDP.
Comparison: Baseline Scan (06/10/2025) vs. Follow-up Scan (2026)
Findings
Skeletal Metastases:
Multifocal, intense abnormal radiotracer uptake is identified throughout the axial and appendicular skeleton, consistent with widespread osseous metastatic disease:

- **Axial Skeleton:** Diffuse involvement of the skull (mixed pattern with prominent occipital uptake), sternum, multiple bilateral ribs, entire spine, and pelvic bones.
- **Appendicular Skeleton:**
  - Bilateral femoral involvement is noted. **New finding:** A prominent, intensely avid lesion in the left femoral mid-shaft.
  - New involvement of the proximal left tibia.
  - Persistent uptake in the right shoulder.

Non-Skeletal Findings:

- Symmetrical, exaggerated periarticular tracer accumulation at both knees and ankles, suggestive of reactive changes or degenerative arthropathy.

Conclusion (considering the prior bone scan)

- Widespread Progressive Skeletal Metastatic Disease: Disseminated osteoblastic and mixed sclerotic lesions are stable in distribution but demonstrate **progressive involvement**, with new conspicuous lesions in the left femoral mid-shaft and proximal left tibia.
- Clinical Correlation: The prominent left femoral mid-shaft lesion is of particular concern due to an elevated risk of pathological fracture. Orthopedic consultation and consideration of prophylactic intervention or targeted imaging (e.g., CT or MRI) is strongly recommended.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

DIAGNOSTIC CENTER

Call Center 19656

Bone Scan
[[DOCTYPE:BONE_SCAN]]

NILE
SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Faten Hamama Mohammed Shehata	Date : 06-10-2025 02:21
Branch : Dokki	Medical No. : 219366
Ref.Physician Prof : Gostaf Roseh Hospital	

$Tc^{99}m$ MDPSKELETAL SCINTIGRAPHY

History

- A 65-year-old patient, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Widespread foci of enhanced tracer uptake are seen at the skull, sternum, multi-levels of the spine, both scapulae, both humeri, bilateral ribs, pelvic bones, both femora, both knees and ankles.
- Degenerative changes are noted at the spine and big joints.

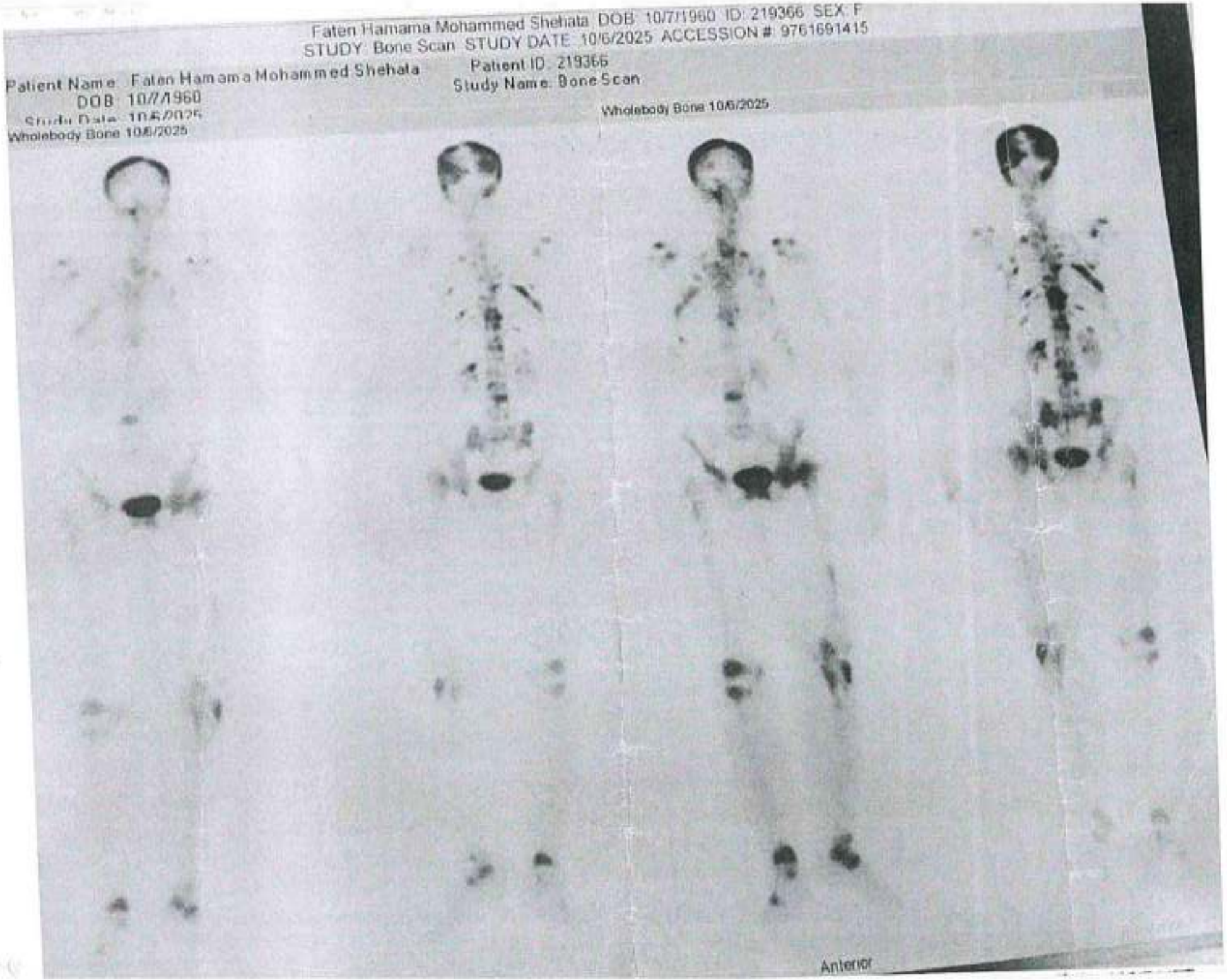
Impression:

- Widespread metastatic osseous lesions as described.

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]


Faten Hamama Mohammed Shehata DOB 10/7/1960 ID: 219366 SEX: F
STUDY Bone Scan STUDY DATE 10/6/2025 ACCESSION #: 9761691415
Patient Name: Faten Hamama Mohammed Shehata
DOB: 10/7/1960
Patient ID: 219366
Study Name: Bone Scan
Wholebody Bone 10/6/2025
Wholebody Bone 10/6/2025
Anterior

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستاف روسي

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

الست المصرية هتاخد صحة مصر

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	**MRN:**	6246
Age:	66 Y	**ID:**	26004242300444
Menopausal Sta		**Phone:***	01110471005
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	mbc triple positive with query DP
Surgical History:* -		**ECOG Performance Status**	○ 0 ○ 1 ○ 2 ◉ 3 ○ 4

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** 3+ **KI67:**

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss lesions pt rec pall rth to left hip j si j cts 5/5/2026 rd for new bone scan
pt rec 7 cycles herceptin 21 weeks taxol
Patient couldn't receive palliative radiotherapy as she is afraid as she took on the same site before

Last MDT > bone scan on 18/6/2026 widespread osteoblastic and sclerotic osseous lesions >> shift to Tykerb- Xeloda + Zometa monthly

for reMDT as the CTs show a stationary course while the BS shows query progressive disease in left midshaft femora + proximal tibia + fibula while old BS shows uptake in both femoral bones

pt did not receive any xeloda tykerb despite approvals and has continued herceptin iv to not stop approvals till MDT decision

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPTINE IV / FEMARA
UROLOGY FOR ET HYDRONROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د محمد كيلاني	د محمد كراوي		

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	**MRN:**	6246
Age:	66 Y	**ID:**	26004242300444
Menopausal Sta		**Phone:***	01110471005
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	mbc triple positive with query DP
Surgical History:* -		**ECOG Performance Status**	○ 0 ○ 1 ○ 2 ◉ 3 ○ 4

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** 3+ **KI67:**

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss lesions pt rec pall rth to left hip j si j cts 5/5/2026 rd for new bone scan pt rec 7 cycles herceptin 21 weeks taxol

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for reMDT as the CTs show a stationary course while the BS shows query progressive disease in left midshaft femora + proximal tibia + fibula while old BS shows uptake in both femoral bones

pt did not receive any xeloda tykerb despite approvals and has continued herceptin iv to not stop approvals till MDT decision

Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPTINE IV / FEMARA
UROLOGY FOR ET HYDRONROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د محمد كيلاني	د محمد الفراوي		

الهيئة العامة للمستشفيات والمعاهد التعليمية
مستشفى احمد ماهر التعليمى
The General Organization for Teaching Hospitals
and Institutes
Ahmad Maher Teaching Hospital

To whom it may concern

Mrs./ Mr ..... years old

female/ male patient presented with diagnosis .....

.....

She/ He was referred to us for (radical / adjuvant / Neoadjuvant / palliative) Radiotherapy

She/ He has received (3D/ IMRT) radiotherapy to.....

.....

.....

At Dose : ..Gy/..fx/..w .

Additional (Boost) ; yes/ no at dose .....Gy/.....fx/.....w

(sequential / concomitant)

Started: ../../2026. 30/9/2025

Ended: ../../2026 With good tolerance .

Head of department
Dr. Hany William

E-mail: Amaher.gothi.com 23914455: ف -23911838- 23913366 : ت القاهرة – باب الخلق – شارع بور سعيد 3411

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مستشفى احمد ماهر التعليمى
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She / He has received (3D/ IMRT) radiotherapy to.....
Rt hip + SIJ

At Dose :20 Gy/ 5 fx/ 1 w .

Additional (Boost) ; yes/ no: at dose .....Gy/.....fx/.....w
(sequential / concomitant)

Started: 30 /9 /2025.

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جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

08/30/2026

MDT FORM

Name:	Faten Hamama Mohamoud Shehata	**MRN:**	6246
Age:	66 Y	**ID:**	26004242300444
Menopausal Sta		**Phone:***	01110471005
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	mbc triple positive with query DP
Surgical History:* -		**ECOG Performance Status**	○ 0 ○ 1 ○ 2 ◉ 3 ○ 4

Clinical Data:

BX:

ER: 8/8 **PR:** 8/8 **HER2:** 3+ **KI67:**

History:

postm bilateral breast cancer met to bone rt er7/8 pr6/8 her2-ve left er8/8 pr8/8 her23+ rec taxol herceptin initial bs 10/2.025 widespread oss lesions pt rec pall rth to left hip j si j cts 5/5/2026 rd for new bone scan pt rec 7 cycles herceptin 21 weeks taxol

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Sonomammography:

Biomarker/Molecular

Mets Workup:

Recommendations:

MDT Outcome & Plan

STATIONARY DISEAS FOR NEW TUMOR MARKER CT CAP CONTINUE
HERCPTINE IV / FEMARA
UROLOGY FOR ET HYDRONROHROSIS
ZOMETA/3 MONTHS
PORTA-CTH
ECHO

R L



T N M

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د محمد كيلاني	د محمد الفراوي		

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